

# SRA-001 Module 1: Introduction to Hijama

## Instructor Guide

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### Module at a Glance

| Attribute               | Detail                                                         |
|-------------------------|----------------------------------------------------------------|
| <b>Module</b>           | M1 —Introduction to Hijama                                     |
| <b>Duration</b>         | 8 hours (2 lessons x 4 hours)                                  |
| <b>Lessons</b>          | 1.1: What is Hijama? / 1.2: The Hijama Practitioner            |
| <b>Slides</b>           | 16 slides                                                      |
| <b>Learning Domains</b> | K1, K2, I1, I2                                                 |
| <b>Prerequisites</b>    | None                                                           |
| <b>Equipment</b>        | Projector, whiteboard, sample cupping cups (for demonstration) |

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### Learning Outcomes

Upon completion, students will be able to:

1. Define Hijama (wet cupping) with clinical precision
2. Distinguish between wet cupping, dry cupping, and other modalities
3. Explain the regulatory status of cupping therapy in key jurisdictions
4. Articulate the professional identity and scope of practice of a CHP
5. Apply the concepts of amanah and ihsan to clinical practice
6. Describe the SRA Competency Framework and its six domains

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### Lesson 1.1 — What is Hijama? (4 hours)

#### Timing Breakdown

| Segment                   | Duration | Activity                                      |
|---------------------------|----------|-----------------------------------------------|
| Opening                   | 10 min   | Welcome, niyyah, module overview              |
| Learning Objectives       | 5 min    | Review outcomes, prior knowledge check        |
| Core Teaching: Definition | 45 min   | Slides 4-6: Definition, modalities, framework |
| Break                     | 15 min   |                                               |

Table 2 – continued

| Segment                   | Duration | Activity                                            |
|---------------------------|----------|-----------------------------------------------------|
| Core Teaching: Regulatory | 45 min   | Slides 7-8: Regulatory landscape, scope of practice |
| Clinical Scenario         | 30 min   | Case study: The Uncertain Patient                   |
| Knowledge Check           | 15 min   | Quiz and discussion                                 |
| Practical Activity        | 30 min   | Examine cupping equipment, identify modalities      |
| Reflection                | 10 min   | Amanah reflection prompt                            |
| Summary                   | 5 min    | Key takeaways, preview Lesson 1.2                   |

### Teaching Notes by Slide

#### Slide 4: Defining Hijama

- Emphasise the precision of the clinical definition
- Show actual cupping cups and demonstrate the three elements
- Ask students: “Why must the definition be so specific?”
- Key point: Precision in definition = precision in practice

#### Slide 5: Cupping Modalities

- Bring sample cups for each modality if available
- Highlight: Many patients use “cupping” to mean any modality
- Stress the importance of clarifying which modality is being discussed
- Clinical pearl: Always ask “Do you mean wet or dry cupping?”

#### Slide 6: The SRA Definition Framework

- Walk through each of the four elements
- Ask: What happens if we remove the incision element? (Answer: it becomes dry cupping)
- Connect to patient safety: precise definition = clear scope

#### Slide 7: Regulatory Landscape

- Ask students to research their own jurisdiction’s regulations
- Emphasise: regulations change — commit to staying updated
- Key point: SRA standards exceed most jurisdictional minimums

#### Slide 8: Scope of Practice

- This is a critical slide for patient safety
- Role-play exercise: Patient asks for a diagnosis — how do you respond?
- Emphasise referral pathways: “When in doubt, refer”
- Documented scope boundaries protect both patient and practitioner

## Common Student Questions

**Q: Can I practise Hijama without being a doctor?** A: Yes — Hijama is a complementary therapy, not a medical practice. However, you must work within the defined scope, maintain appropriate insurance, and comply with local regulations. You cannot diagnose, prescribe, or claim to cure diseases.

**Q: What's the difference between CHP and a doctor?** A: A doctor diagnoses and treats diseases using medical interventions. A CHP provides Hijama (wet cupping) as a complementary therapy within a defined scope. CHPs do not replace medical care — they complement it.

**Q: Can I refuse to treat someone?** A: Yes — and sometimes you must. If a contraindication exists, if the patient lacks capacity to consent, or if the case is beyond your competence, you must decline treatment and refer appropriately.

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## Lesson 1.2 — The Hijama Practitioner (4 hours)

### Timing Breakdown

| Segment                   | Duration | Activity                                       |
|---------------------------|----------|------------------------------------------------|
| Opening                   | 10 min   | Recap Lesson 1.1, niyyah                       |
| Learning Objectives       | 5 min    | Review outcomes                                |
| Core Teaching: Amanah     | 40 min   | Slides 10-11: Amanah concept and framework     |
| Core Teaching: Ihsan      | 30 min   | Slide 12: Ihsan in clinical practice           |
| Break                     | 15 min   |                                                |
| Core Teaching: Competency | 30 min   | Slide 13: Competency Framework                 |
| Core Teaching: Identity   | 30 min   | Slide 14: Professional identity and boundaries |
| Clinical Scenario         | 30 min   | Case study: The Boundary Test                  |
| Knowledge Check           | 15 min   | Quiz and discussion                            |
| Practical Activity        | 20 min   | Self-assessment against Competency Framework   |
| Reflection                | 10 min   | Ihsan reflection prompt                        |
| Summary                   | 5 min    | Module summary, preview Module 2               |

### Teaching Notes by Slide

#### Slide 10: The Practitioner as Amanah-Holder

- This is the spiritual foundation of the programme
- Read the Qur'anic verse aloud (33:72)
- Ask: "What does it mean to hold a sacred trust?"
- Share a personal story about trust in clinical practice
- Key point: Every patient is an amanah — this transforms how we practise

#### Slide 11: The SRA Amanah Framework

- Walk through each pillar with clinical examples
- Exercise: Students rate themselves 1-5 on each pillar
- Discussion: Which pillar is hardest to maintain? Why?
- Connect to assessment: The Amanah Framework appears in OSCE scenarios

#### **Slide 12: Ihsan in Clinical Practice**

- Read the Hadith of Gabriel aloud
- Explain: Ihsan is not perfectionism — it is conscious excellence
- Discussion: “What does ihsan look like in a treatment room?”
- Key point: Ihsan transforms a technical procedure into worship

#### **Slide 13: The SRA Competency Framework**

- Explain the six domains and four levels
- Students self-assess their current level in each domain
- Emphasise: Level 3 is required for certification — this is rigorous
- Connect to assessment: Each domain is tested in written exam and/or OSCE

#### **Slide 14: Professional Identity and Boundaries**

- Role-play: Maintaining boundaries with a difficult patient
- Discuss interprofessional relationships and referral pathways
- Key point: Professional boundaries protect both patient and practitioner
- Emphasise: “You cannot pour from an empty vessel” — self-care is professional duty

#### **Common Student Questions**

**Q: Do I need to be Muslim to practise Hijama?** A: No — the SRA programme is open to all. However, you must understand and respect the Islamic foundations of Hijama, as patients may ask about them. The ethical principles (amanah, ihsan) are universal values that apply to all practitioners.

**Q: How do I handle a patient who wants me to treat their child?** A: Children require special consideration. In most jurisdictions, parental consent is required for minors. Additional training in paediatric considerations is recommended. When in doubt, refer to the patient’s GP.

**Q: What if I make a mistake?** A: Mistakes happen — how you respond defines your professionalism. Acknowledge the error, document it, inform the patient if appropriate, seek guidance, and learn from it. The Ihsan Quality Cycle (SRA-CEF-001) provides a framework for continuous improvement.

### **Assessment Guidance**

#### **Formative Assessment (During Module)**

#### **Knowledge Check Questions (Slide 15):**

1. b) Controlled incision + negative pressure + blood extraction

2. c) Perform wet cupping within scope
3. c) Level 3: Competent
4. b) Financial Gain

#### **Case Study 1 — The Uncertain Patient:**

- Look for: clear communication, accurate distinction between modalities, informed consent process, honest expectations
- Common error: Students may over-promise outcomes or fail to distinguish modalities

#### **Case Study 2 — The Boundary Test:**

- Look for: recognition of scope boundary, ethical response, appropriate referral, maintenance of therapeutic relationship
- Common error: Students may either agree inappropriately or respond too harshly

#### **Summative Assessment (Module 1 Content in Final Exam)**

Module 1 content will appear in:

- **Written Examination:** Multiple choice questions on definitions, scope, and ethics
  - **OSCE Station 2 (Consent and Communication):** Applying amanah and ihsan principles
  - **Portfolio:** Demonstrating professional identity and ethical practice in case documentation
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### **Practical Activities**

#### **Activity 1: Equipment Identification (Lesson 1.1)**

**Materials:** Sample cups (plastic, glass, silicone), pump, lancets, gloves

**Instructions:**

1. Display various cupping equipment
2. Students identify each type and its modality
3. Students explain which equipment is used for wet vs. dry cupping
4. Discuss: Why might you choose one cup type over another?

**Learning Outcome:** Students can distinguish equipment by modality

#### **Activity 2: Competency Self-Assessment (Lesson 1.2)**

**Materials:** Competency Framework handout

**Instructions:**

1. Students rate themselves 1-4 in each domain
2. Students identify their strongest and weakest domains
3. Students write one action for improving their weakest domain
4. Share (voluntarily) with a partner for accountability

**Learning Outcome:** Students understand the competency requirements and their personal development needs

### Activity 3: Boundary Role-Play (Lesson 1.2)

**Materials:** Scenario cards

**Instructions:**

1. In pairs, one student plays practitioner, one plays patient
2. Patient presents a request beyond scope (diagnosis, prescription, etc.)
3. Practitioner responds maintaining professionalism and relationship
4. Debrief: What was effective? What was challenging?

**Learning Outcome:** Students can maintain professional boundaries with compassion

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## Differentiation Strategies

### For Students Without Healthcare Background

- Provide additional anatomy terminology glossary
- Allow extra time for regulatory research exercise
- Pair with students who have clinical experience

### For Students With Prior Clinical Experience

- Ask them to share relevant clinical stories (anonymised)
- Encourage them to mentor less experienced peers
- Challenge them to connect Hijama to their existing knowledge base

### For Students for Whom English is a Second Language

- Provide key terms in advance
  - Allow use of dictionaries during exercises
  - Speak clearly and check for understanding
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## Islamic Integration Notes

### Key Islamic References in This Module

| Reference                          | Location    | Teaching Approach                                       |
|------------------------------------|-------------|---------------------------------------------------------|
| Qur' an 33:72 (al-Amanah)          | Slide 10    | Read aloud, discuss meaning, connect to clinical ethics |
| Qur' an 4:58 (Rendering trusts)    | Final slide | Reflection prompt, personal commitment                  |
| Sahih Muslim 8 (Hadith of Gabriel) | Slide 12    | Explain context, discuss application to practice        |

Table 4 – continued

| Reference             | Location | Teaching Approach                    |
|-----------------------|----------|--------------------------------------|
| Sahih al-Bukhari 5371 | Slide 4  | Authentic Hadith evidence for Hijama |

### Sensitivity Notes

- Not all students may be Muslim — present Islamic principles as the foundation of SRA practice while respecting all faiths
- Use Islamic references to enrich understanding, not to exclude
- Encourage questions about Islamic dimensions — they are part of the curriculum

### Troubleshooting

| Issue                                        | Solution                                                                                         |
|----------------------------------------------|--------------------------------------------------------------------------------------------------|
| Student asks for medical advice during class | Remind them you are an educator, not their clinician. Refer to their GP.                         |
| Student challenges scope boundaries          | Use the Clinical Decision Tree. Ask: “What would happen if a CHP diagnosed and got it wrong?”    |
| Student expresses overconfidence             | Reference the Competency Framework. Overconfidence is a safety risk.                             |
| Student expresses anxiety about competence   | Normalise it. All practitioners start at Level 1. The programme builds competence progressively. |
| Technical issue with slides                  | Have slides available in PDF as backup.                                                          |

### Post-Module Checklist

- All 16 slides presented
- Both case studies discussed
- All knowledge check questions answered
- Practical activities completed
- Reflection prompts addressed
- Student questions documented
- Attendance recorded
- Any concerns about individual students noted
- Feedback collected

### Contact

**Academic Director:** [academic.director@sunnahremedies.academy](mailto:academic.director@sunnahremedies.academy) **Technical Support:** [support@sunnahremedies.academy](mailto:support@sunnahremedies.academy) **Emergency (student welfare):** [welfare@sunnahremedies.academy](mailto:welfare@sunnahremedies.academy)

*Sunnah Remedies Academy — Institute of Prophetic Medicine*

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